

PATIENT INFO

Patient Name: Rowan Quillstone (fictional synthetic patient)

Age: 47 years

Sex: Female

Medical Record Number: SYN-48291-XF (synthetic identifier)

Date of Admission: 14 March 2026

Date of Discharge: 18 March 2026

Admitting Service: General Internal Medicine

Primary Care Provider: None listed

Allergies: Rash with cefaclor; no known food allergies

Past Medical History: Essential hypertension, type 2 diabetes mellitus, hyperlipidemia, mild intermittent asthma

Chief Complaint: Four-day history of productive cough, fever, fatigue, and worsening shortness of breath

Hospital Course: Chest imaging and clinical findings were consistent with right lower lobe community-acquired bacterial pneumonia. The patient received intravenous antibiotics, supplemental oxygen, intravenous fluids, bronchodilator therapy, and supportive care. Clinical status improved steadily, oxygen therapy was discontinued prior to discharge, and the patient was discharged home in stable condition.

DIAGNOSIS

Primary Diagnosis: Community-acquired bacterial pneumonia involving the right lower lobe

Secondary Diagnosis: Acute hypoxemic respiratory insufficiency, resolved

Secondary Diagnosis: Type 2 diabetes mellitus with suboptimal glycemic control

Secondary Diagnosis: Essential hypertension

Secondary Diagnosis: Hyperlipidemia

Secondary Diagnosis: Mild intermittent asthma without acute exacerbation

Clinical Outcome: Symptoms significantly improved with treatment and no inpatient complications occurred

MEDICATIONS

Amoxicillin-Clavulanate: 875 mg/125 mg orally twice daily for 5 days

Azithromycin: 500 mg orally once daily for 2 days

Metformin Extended Release: 1000 mg orally twice daily with meals

Empagliflozin: 10 mg orally once daily

Lisinopril: 20 mg orally once daily

Atorvastatin: 40 mg orally at bedtime

Albuterol Inhaler: 90 micrograms per actuation, inhale 2 puffs every 6 hours as needed for wheezing or shortness of breath

Acetaminophen: 650 mg orally every 6 hours as needed for pain or fever; maximum 3000 mg per 24 hours

LAB RESULTS

Test	Result	Reference Range
White Blood Cell Count	8.4 x10 ⁹ /L	4.0-11.0 x10 ⁹ /L
Hemoglobin	13.1 g/dL	12.0-16.0 g/dL
Hematocrit	39.5%	36.0-46.0%
Platelet Count	286 x10 ⁹ /L	150-400 x10 ⁹ /L
Sodium	139 mmol/L	135-145 mmol/L
Potassium	4.2 mmol/L	3.5-5.1 mmol/L
Chloride	103 mmol/L	98-107 mmol/L
Bicarbonate	25 mmol/L	22-29 mmol/L
Blood Urea Nitrogen	16 mg/dL	7-20 mg/dL
Creatinine	0.88 mg/dL	0.60-1.20 mg/dL
Estimated Glomerular Filtration Rate	91 mL/min/1.73 m ²	>60 mL/min/1.73 m ²
Glucose	146 mg/dL	70-99 mg/dL fasting
Aspartate Aminotransferase (AST)	24 U/L	10-40 U/L
Alanine Aminotransferase (ALT)	27 U/L	7-56 U/L
Alkaline Phosphatase	82 U/L	44-147 U/L
Total Bilirubin	0.7 mg/dL	0.2-1.2 mg/dL
Albumin	3.9 g/dL	3.5-5.0 g/dL
C-Reactive Protein	12.4 mg/L	<5.0 mg/L
Peak C-Reactive Protein During Admission:	98.7 mg/L	
Procalcitonin	0.18 ng/mL	<0.25 ng/mL
Blood Culture: No growth after 5 days		
Respiratory Viral PCR Panel: Negative for common respiratory viruses		

VITALS

Temperature: 36.8 degC

Blood Pressure: 126/78 mmHg

Heart Rate: 74 beats/minute

Respiratory Rate: 16 breaths/minute

Oxygen Saturation: 97% on room air

Height: 168 cm

Weight: 74.6 kg

Body Mass Index: 26.4 kg/m²

Pain Score: 1/10

DISCHARGE INSTRUCTIONS

Complete the prescribed antibiotic course without missing doses.

Continue all chronic medications as prescribed.

Maintain adequate oral hydration and resume normal physical activity gradually as tolerated.

Monitor blood glucose regularly and follow a diabetes-appropriate diet.

Monitor blood pressure at home at least once daily for the next week.

Schedule follow-up with a primary care clinician within one week after discharge.

Repeat chest imaging in approximately six weeks if recommended by the treating clinician.

Seek immediate medical attention for persistent fever, worsening shortness of breath, chest pain, significant coughing of blood, confusion, inability to tolerate medications, or any rapidly worsening symptoms.

This discharge summary is entirely fictional and synthetic and is intended solely for testing document-processing and information-extraction systems.